

PRODUCT LITERATURE

DIMERINE SYRUP 25MG/5ML

Each 5 ml contains:

Dimenhydrinate 25.00 mg

Preservatives:

Methyl Paraben 0.1%w/v
Propyl Paraben 0.01%w/v

Description

Clear colourless syrup with fruity odour and sweet taste.

Pharmacodynamic properties

Dimenhydrinate is the salt produced by interaction of the antihistaminic base diphenhydramine with the acidic compound 8-chlorotheophylline. Dimenhydrinate markedly depresses labyrinthine function. Because of the receptors with which it interacts, Dimenhydrinate is described as an H¹-antagonist or the blocker of histamine and belongs to the Theanolamine group. The mode of action is a result of the binding with high affinity to H¹-receptors in the brain. It is not, however, clear whether the anti-motion sickness activity of Dimenhydrinate is related to its ability to block muscarinic receptors.

Pharmacokinetic properties

Dimenhydrinate is well absorbed from the gastro-intestinal tract after oral dosing with extensive first-pass effect. The drug is metabolised in the liver and excreted usually as metabolites in the urine. The drug is highly bound to plasma proteins and is widely distributed in the body. Following oral administration, the effects develop in about 30 minutes and are maximal within 1-2 hours and last for 3-6 hours.

Indication

Management of nausea and vomiting of motion sickness and vertigo-associated diseases affecting the vestibular system.

Route of administration

Oral.

Recommended Dose

To be taken 3 to 4 times a day.
2 - 6 years: 1/2 - 1 teaspoonful (2.5 ml - 5 ml)
6 - 12 years: 1 - 2 teaspoonfuls (5 ml - 10 ml)
Adult: 2 - 4 teaspoonful (10 ml - 20 ml),
Motion sickness: To be taken 1 hour before journey starts

Contraindication

Sensitivity to Dimenhydrinate or any of the other ingredients of the tablet.

Warning and Precautions

Dimenhydrinate should be used with caution in epilepsy, prostatic hypertrophy, glaucoma and hepatic diseases. It has been suggested that Dimenhydrinate could mask warning symptoms of damage caused by ototoxic drugs such as the amino-glycoside antibiotics.

Interactions with Other Medicaments

Dimenhydrinate will interact with anticholinergic and anti-parkinsonian drugs such as Benzhexol, increasing the anticholinergic side effects, dry mouth, urine retention, confusion, etc. Patients should be warned not to take alcohol while under treatment with drugs affecting the Central Nervous System, in particular anti-histamines such as Dimenhydrinate. Because of the potentiating threat of Dimenhydrinate on the action of the Central Nervous System depressants it is important that the dose of Neperidine, Morphine or other narcotic analgesics and of barbiturates be reduced by ¼ or ½ when used concomitantly.

Pregnancy and Lactation

Dimenhydrinate should not be used in pregnancy unless the physician considers it is essential. There was a significant incidence of cleft palate and clefts with other defects in children whose mothers have taken diphenhydramine (a component of Dimenhydrinate).

Side Effects

Adverse effects with Dimenhydrinate may vary in incidence and severity from patient to patient. The most common effect is sedation which may vary from slight drowsiness to deep sleep. The drug may be associated with inability to concentrate, lassitude, dizziness, hypotension, muscular weakness and uncoordination. When they do occur the sedative effects may diminish after a few days. Rare with Dimenhydrinate are gastro-intestinal side effects. Dimenhydrinate may very rarely produce headache, blurred vision, tinnitus, elation or depression, irritability, nightmares, anorexia, difficulty in micturition, dryness in the mouth, tightness in the chest, tingling, heaviness and weakness of the hands. Although cardio-vascular side effects are rare, minor increases in blood pressure and occasional mild hypotension have been reported. Leucopenia and rarely agranulocytosis, jaundice and extra-pyramidal reactions have also been reported. Occasionally hypersensitivity reactions have followed its uses by both mouth and topical application.

Symptoms and Treatment of Overdose

In the case of severe overdosage, the stomach should be emptied by gastric lavage. Emetics should not be used. The patient should be kept quiet, particularly in the case of children, to minimise the excitation which occurs. Convulsions may be controlled with Diazepam preferably given intravenously. Since Dimenhydrinate is rapidly metabolised with only traces being recoverable in the urine, diuresis is of little, if any, value.

Packing

Packed in plastic container of 100 ml.

Storage Conditions

Store below 30°C in a dry place, protected from direct light.
Keep away from reach of children.
Jauhkan daripada kanak-kanak

CONTROLLED MEDICINE/UBAT TERKAWAL

Expiry Date

2 years from date of manufacture.

Product Registration Holder / Distributor:

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